NCCP 00947 · VERSION 1

Capivasertib + fulvestrant

Capivasertib is taken twice daily for four days, then stopped for three days, each week. Fulvestrant injections are given on Days 1 and 15 of the first 28-day cycle, then Day 1 of later cycles. Treatment continues while helpful and tolerated.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Diarrhoea, dehydration and salt imbalance

☐ Rash, itch or severe skin reactions

☐ Changes in blood counts or kidney/liver tests, assessed by the team

☐ Hot flushes and joint or muscle discomfort

☐ Allergic reactions; discuss pregnancy prevention and fertility

☐ Raised blood sugar or diabetes; rarely a dangerous ketone/acidity problem called diabetic ketoacidosis

☐ Nausea, vomiting, sore mouth, reduced appetite and tiredness

☐ Pain, bruising or bleeding at the injection sites; rarely nerve injury symptoms

☐ Nausea, tiredness and changes in liver tests

Discuss glucose/diabetes and ketoacidosis, diarrhoea/dehydration, skin reactions and injection risks. Agree the tablet pattern, glucose plan and contact route. Pregnancy and breastfeeding are excluded. Contraception advice must cover the longer fulvestrant period: two years after its last dose for women; capivasertib male advice extends sixteen weeks after stopping. Discuss fertility and the individual plan.

Individual benefit, alternatives and concerns

Agreed checks and next review

Check blood count, kidney and liver function before treatment and each cycle. Check fasting glucose and HbA1c before starting. Fasting glucose is checked at weeks 1, 2, 4, 6 and 8, then monthly; ideally before the tablet dose on day 3 or 4 of a dosing week. HbA1c is checked every three months, with an extra week-4 check for diabetes, prediabetes or high baseline glucose. Diabetes, new high glucose or metformin initiation needs extra checks under a written plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.20 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending